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LIME EMR · TESTING

# Test cases: examples

How the MSF metadata workbook becomes automated tests, with nothing new to fill in

## What is a test case?

A test case is one sentence: if I do this, I should see that. It names who does it (role, site) and gives a precise, observable result. Every rule already written in the metadata workbook is a test case; we check it in the EMR automatically and report the result in the Form QA Tracker.

## 1. Form rules: already covered, nothing to do

Each Skip logic, Mandatory and Lower/Upper limit entry of a form tab gives one or two test cases. Examples from F17 Surgery Admission and F48 Blood Transfusion:

| What the form tab says | Test case generated | Result |
|---|---|---|
| Skip logic: Hide question if [Referred to MSF] !== 'Yes' (on « By whom / from where ») | I answer « Yes » to « Referred to MSF » → « By whom / from where » appears. I answer « No » → it does not appear. | ✅ Matches |
| Lower limit: 0 (on « Weight at admission (in kg) ») | I enter -1 → the value is refused. | ✅ Matches |
| Skip logic: Hide question if [Reactions] !== 'Fever', …, 'Other' (F48, « Management of reactions ») | I tick « Other » in Reactions → « Management of reactions » appears. | ❌ Difference: the form does not show it for « Other » |

Across the Release 1 workbook: about 3,900 test cases generated, 91% matching the EMR. Each difference is reviewed together: either the form or the tab is updated.

## 2. Another test on one question: the Comments column

For what the columns cannot express (a calculation, a button, a missing limit), write one line per test in the existing Comments column of the form tab: Test: what I do -> what I should see.

| Tab, question | Comments column |
|---|---|
| F35, PHQ-9 score | Test: Answer « Nearly every day (3 points) » to the 9 questions -> PHQ-9 score = 27 |
| F17, Order medications | Test: Click « Order medications » -> The order basket opens for this patient |
| F17, Weight at admission (in kg) | Test: Enter 450 -> The value is refused |

*OpenMRS 3 forms refuse a value outside the Lower/Upper limit; they cannot show a soft warning. The weight test therefore also asks for an Upper limit, which F17 does not have today.*

## 3. Beyond one question or beyond forms: the User flow template tab

Patient journeys, user privileges and confidentiality, appointments, registration and DHIS2 go in the existing User flow template tab, one step per row. Step starts with a short code; Action is what I do; Description is what I should see. User is the EMR role (workbook role name in brackets). ROLES-2 and ROLES-3 come from the User roles and privileges tab and show two differences to review together.

| Step | Action | Description (what I should see) | User | Today |
|---|---|---|---|---|
| F31-1 | Save an F31 mhGAP Baseline during an active visit, then open the patient chart | The form appears under the visit in the Visits tab and in Forms & Notes | Mental Health Counselor (MH counselor) | To test |
| ROLES-1 | Open the patient chart of a mental health patient | Mental health forms are not visible | Registration Officer (Registration Clerk) | ✅ Config matches |
| ROLES-2 | Open an F31 mhGAP Baseline, go to Clinical diagnosis | The section is read-only without the « Edit sensitive mental health fields » right | Mental Health Nurse | ❌ Not locked in F31 (was in F03) |
| ROLES-3 | Open the order basket, try to order a lab test | No lab order option | Mental Health Counselor (MH counselor) | ❌ The EMR role can add orders |
| APPT-1 | Book a follow-up appointment | One service per counselor: Mental Health - AB, DT, KA, SA, SK | Registration Officer | To test |
| REG-1 | Register a new patient | An MSF ID is generated automatically: prefix IQ146-, at most 16 characters | Registration Officer | ✅ Config matches |
| DHIS2-1 | Save an F35 PHQ-9, wait for the daily DHIS2 sync | The PHQ-9 score appears in DHIS2 for that patient | Automatic (daily sync) | To test |

## Writing a good test

- **Precise:** « PHQ-9 score = 27 », not « the score is correct ». One test checks one thing.
- **Negative cases:** Also write what must not happen: « Clinical diagnosis is read-only for the Mental Health Nurse », not « confidentiality works ».
- **Field issues:** A problem seen in the field becomes a test, so it does not come back.
- **Data:** Never use real patient data in a test.

## What would help us most

- Keep filling the form tabs as today, following the Form Guidance tab.
- Add tests in the Comments column and the User flow template tab when a rule is not covered by the columns.
- Tell us your priority areas (for example confidentiality, appointments, DHIS2), and the EMR role matching MedCo, MH manager and Data officer.